

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

October 16, 2025

MEMORANDUM FOR:

(b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

CAPT (b)(6),(b)(7)(C),(b)(7)(F) MD, MPH, CCHP, FAAFP
Chief Medical Officer
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CDR (b)(6),(b)(7)(C),(b)(7)(F) RN
Chief, Investigations Unit
ICE Health Service Corps

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FROM:

LCDR (b)(6),(b)(7)(C),(b)(7)(F) RN
Investigator, Investigations Unit
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT:

Mortality Review – Report of Findings
Tien Xuan PHAN, (b)(6),(b)(7)(C)

Executive Summary:

On July 20, 2025, the U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of alien Tien Xuan PHAN, (b)(6),(b)(7)(C) ICE detained Mr. PHAN, a 55-year-old Vietnamese male, at Karnes County ICE Processing Center (KCIPC), in Karnes City, Texas (TX), from June 2 to July 19, 2025, the date of his death. A physician determined Mr. PHAN's cause of death was intracranial hemorrhage.

In December of 1999, Mr. PHAN obtained the status of a lawful permanent resident of the United States (U.S.) after he entered the U.S. in August of 1990 as a refugee. However, in 2006, a judicial court in the State of Alabama convicted Mr. PHAN of possession of a controlled

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substance and sentenced him to 20 years' confinement with a suspended sentence and five years' probation. Mr. PHAN briefly departed the U.S. in December of 2011, and U.S. Customs and Border Protection apprehended him when he attempted to illegally re-enter the U.S. in March of 2012. In April of 2012, an immigration judge ordered Mr. PHAN's removal to Vietnam and ICE released Mr. PHAN on an order of supervision on July 2, 2012. On June 2, 2025, ICE revoked his order of supervision and detained him at KCIPC.

On June 2, 2025, during his medical intake screening, Mr. PHAN reported a history of diabetes, for which he took oral medications, and "possible" hypertension (high blood pressure). A physician completed his comprehensive health assessment the following day and ordered routine labs, medications to treat his diabetes and elevated blood pressure, and blood pressure and blood glucose monitoring.

On June 11, 2025, upon receipt of his lab results, an advanced practice provider (APP) diagnosed Mr. PHAN with hyperlipidemia (high cholesterol) and ordered medication accordingly. The APP also recommended Mr. PHAN start insulin due to his lab results showing evidence of kidney damage and poor blood glucose control, but he refused to start insulin.

Throughout June, health care staff continued to monitor Mr. PHAN's diabetes and blood pressure, and adjusted his medication regimen and plan of care as clinically indicated.

On July 18, 2025, at 5:25 a.m., medical staff responded to a medical emergency due to Mr. PHAN exhibiting seizure-like activity. Medical staff assessed Mr. PHAN's condition and requested emergency medical services (EMS) at 5:36 a.m. At 5:42 a.m., EMS personnel responded and transported Mr. PHAN to the Otto Kaiser Memorial Hospital (OKMH) emergency department (ED). At the ED, OKMH staff identified Mr. PHAN's frontal parietal brain bleed and intubated him prior to transferring him to Northeast Methodist Hospital (NMH) for a higher level of care. Mr. PHAN's condition continued to rapidly decline, and NMH medical staff pronounced Mr. PHAN deceased on July 19, 2025, at 5:48 p.m.

Mortality Finding:

As a result of Mr. PHAN's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues requiring further study.

A mortality review committee (MRC) determined KCIPC did not provide Mr. PHAN medical care within the safe limits of practice. Specifically, the MRC determined, 1) Medical staff did not

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effectively transition Mr. PHAN's medications from self-administration to pill line, which created a risk of overdose and continued noncompliance; 2) A provider did not continue Mr. PHAN's blood glucose checks after the initial order ended, despite his continued high glucose readings.

The MRC convened on September 24, 2025, and consisted of the following IHSC members: Lieutenant Commander (LCDR) (b)(6),(b)(7)(C),(b)(7)(F) medical doctor, clinical director, physician reviewer; Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) physician assistant, chief APP, APP reviewer; and CAPT (b)(6),(b)(7)(C),(b)(7)(F) registered nurse (RN), western regional nurse manager, nurse reviewer.

The following summary of events was based on: 1 Mr. PHAN's KCIPC medical records; 2) Mr. PHAN's EMS report; 3) incident and notification reports; 4) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 5) Mr. PHAN's KCIPC detention file; 6) video footage from July 18 to 19, 2025; 7) an on-site review and staff interviews conducted by LCDR (b)(6),(b)(7)(C),(b)(7)(F) at KCIPC, from August 19 to 21, 2025; 8) applicable KCIPC policies and procedures; and 9) applicable accreditation standards.

Summary of Events

On June 2, 2025, a licensed practical nurse (LPN) completed Mr. PHAN's medical intake screening and documented he reported a medical history of diabetes and "possible" hypertension, a surgical history of a kidney stone removal (2012), and a prescription for an unknown diabetes medication, but no insulin usage or medications for hypertension. The LPN documented Mr. PHAN's normal vital signs, except for an elevated blood pressure of 148/90 millimeters of mercury [(mm Hg); normal: less than 120/80 mm Hg], and a high blood glucose level of 275 milligrams per deciliter [(mg/dL); normal: 70 – 90 mg/dL]. The LPN submitted a provider referral for the next day, ordered blood pressure checks twice daily, submitted a dental referral for cavities, obtained refusal forms for a COVID-19 vaccine and sexually transmitted disease testing, and cleared him for general population.

On June 3, 2025, a physician completed Mr. PHAN's physical assessment and documented a normal physical exam, mental health screening, diabetic foot exam, and vital signs, except for a blood pressure reading of 162/95 mm Hg. The physician ordered a chest x-ray and electrocardiogram, with normal results, a diabetic diet, and baseline lab work, including a complete metabolic profile (CMP), complete blood count, lipid panel, hemoglobin (Hgb) A1C, thyroid panel, urinalysis, and hepatitis panel. The physician noted Mr. PHAN had poor diabetic and hypertension control, and ordered the following: aspirin (cardiac risk reduction in diabetes) 81 milligrams (mg), once daily, lisinopril (lowers blood pressure) 10 mg, once daily, metformin (diabetes medication) 1,000 mg, twice daily, blood glucose level checks twice daily for 15 days, and continued blood pressure checks twice daily.

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On June 4, 2025, an LPN documented Mr. PHAN received all his medication to keep in his possession for self-administration.

On June 11, 2025, an APP evaluated Mr. PHAN for his chronic care follow-up appointment, reviewed his lab work with him, and explained his cholesterol levels were elevated at 224 mg/dL (normal: <200 mg/dL), low density lipoprotein at 134 mg/dL (normal: ≤ 100 mg/dL), triglycerides at 206 mg/dL (normal: <150 mg/dL), and Hgb A1C at 10.7% (normal: <5.7%). The APP recommended Mr. PHAN start insulin due to his uncontrolled diabetes, high glucose readings (ranging 208 – 409 mg/dL), and his urine containing glucose, protein, and albumin (signs of kidney damage), but Mr. PHAN refused to take insulin because he did not want an injection every day. The APP explained Mr. PHAN's high risk for cardiovascular events due to his diagnosis of hypertension, hyperlipidemia (high cholesterol), and uncontrolled diabetes, and encouraged him to take insulin. The APP ordered atorvastatin (lowers cholesterol) 20 mg, once daily, follow-up lab work, including a lipid panel, CMP, and thyroid stimulating hormone in six weeks, and told Mr. PHAN to return to the clinic if he changed his mind about starting insulin.

On June 13, 2025, an LPN gave Mr. PHAN a 30-day supply of atorvastatin to keep in his possession for self-administration.

- On the same date, an LPN documented Mr. PHAN's blood glucose reading of 444 mg/dL and received a verbal order, from a physician, to give a one-time dose of regular insulin (immediately reduces blood sugar levels), 10 units, now, and add glyburide (diabetes medication) 5 mg, once daily, to his ordered medications.

On June 20, 2025, an APP evaluated Mr. PHAN for a follow-up appointment for his blood pressure and blood glucose readings. The APP noted Mr. PHAN's blood pressure was within normal limits, but his glucose readings remained elevated (ranging from 209 – 444 mg/dL), and he still refused to take insulin. The APP documented Mr. PHAN did not take the morning medications in his possession correctly (after medical staff checked his pill packs for compliance) and ordered his aspirin and lisinopril medications to be placed on pill line but continued his diabetic and cholesterol medications to keep in his possession for self-administration. The physician's order for Mr. PHAN's blood glucose checks ended on June 18, 2025; however, the APP did not continue the order.

On June 24 and 30, 2025, nursing staff documented Mr. PHAN refused his lisinopril and aspirin at pill line and did not list his reason for refusal.

On July 2 and 7, 2025, an LPN gave Mr. PHAN a 30-day supply of aspirin and metformin, and atorvastatin, respectively, to keep in his possession for self-administration.

On July 16, 2025, an LPN documented Mr. PHAN refused to come to pill line to receive his aspirin and lisinopril and did not list his reason for refusal.

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On July 18, 2025, at 5:25 a.m., an RN responded to a medical emergency called in Mr. PHAN's cell, and upon approaching his bed, observed he had vomited, and found him unresponsive and exhibiting seizure activity. The RN placed Mr. PHAN on his side, and the medical and security staff loaded him on a gurney and transferred him to the medical clinic while he continued to vomit intermittently.

- At 5:28 a.m., an RN notified a physician of Mr. PHAN's condition and received orders to transfer him to the emergency room by EMS for evaluation and treatment. The RN documented an elevated heart rate of 114 beats per minute [(BPM); normal: 60 – 99 BPM], blood pressure of 140/70 mm Hg, blood glucose reading of 257 mg/dL, and oxygen saturation of 87% (normal: >95%) on room air. The RN administered oxygen to Mr. PHAN via nasal cannula at 2 liters per minute (oxygen saturation increased to 98%) and positioned Mr. PHAN on his right side until EMS arrived. The RN noted Mr. PHAN's skin appeared warm and dry, and his circulation appeared normal.
- At 5:42 a.m., EMS personnel arrived at KCIPC and assumed care of Mr. PHAN.
- At 6:04 a.m., EMS personnel transported Mr. PHAN (in stable condition) to OKMH, in Kenedy, TX.
- At 10:01 a.m., an RN called the OKMH emergency department, and OKMH medical staff reported Mr. PHAN was in critical condition, intubated and on a ventilator, had suffered a severe brain bleed, and would be airlifted to MHN, in Live Oak, TX.

On July 19, 2025, at approximately 12:00 p.m., MHN medical staff informed ERO San Antonio that Mr. PHAN's next of kin were at the hospital, and they requested withdrawal of all lifesaving measures and the implementation of hospice protocols.

- At approximately 12:21 p.m., MHN medical staff informed ERO San Antonio that Mr. PHAN's family signed a do-not-resuscitate order.
- At approximately 5:31 p.m., MHN medical staff withdrew life-preserving measures and pronounced Mr. PHAN deceased at 5:48 p.m.

The MRC used the following accreditation standards for this review: ICE Performance Based National Detention Standards (PBNDS), 2011 with 2016 revisions, and National Commission on Correctional Health Care (NCCHC) Standards for Health Services in Jails, 2018. The MRC identified the following program strengths, weaknesses, and recommendations during this review.

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Strengths

The MRC identified the following strengths: 1) Medical and custody staff responded to Mr. PHAN's emergency in less than four minutes. Medical staff immediately requested EMS after contacting the physician, and custody staff facilitated EMS personnel's arrival efficiently; 2) The APP displayed excellent documentation for Mr. PHAN's chronic conditions, plan of care, rationale for taking medications for his condition, and the associated risks for refusal. Furthermore the refusal forms contained excellent documentation of the risks for refusing insulin treatment and progression of his disease, and included an option to return to the clinic if he changed his mind; 3) KCIPC completes random hourly wellness checks on all detainees and has procedures in place to monitor medication compliance by requiring that detainees bring all self-administered medications with them to all scheduled clinic appointments.

Weaknesses

1. Inappropriate medical care.

- On June 20, 2025, an APP ordered Mr. PHAN's aspirin and lisinopril for pill line instead of self-administration due to his medication noncompliance; however, medical staff did not discontinue Mr. PHAN's self-administration order. On July 2, 2025, Mr. PHAN received a 30-day supply of aspirin, in addition to the aspirin he received via pill line, which created a risk for overdose. Furthermore, the APP did not change the administration method for Mr. PHAN's diabetic and cholesterol medications to pill line, despite his known noncompliance and insulin refusal.
- On June 4, 2025, a physician ordered Mr. PHAN blood glucose checks for 15 days. On June 20, 2025, an APP evaluated Mr. PHAN for a follow-up appointment and discussed his continued elevated blood glucose readings and need for insulin, but the APP did not notice the expired order for glucose monitoring or continue the blood glucose checks.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section G. *Pharmaceutical Management*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. *Emergency Medical Services and First Aid*.
- NCCHC: Standards for Health Services in Jails, 2018: J-E-02, *Medication Services*.
- KCIPC policy: 13-21, *Management of Diabetes*.
- KCIPC policy: 2-5, *Medication management*.
- KCIPC policy: 1-2, *Medication Administration Training*.

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Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Care Compliance Division (HCCD).
- The IHSC DAD of HCCD will share these findings through appropriate communication channels to ICE, and KCIPC's facility administrator and health authority, for review and any action(s) deemed appropriate.
- The respective IHSC HCCD unit and ICE will ensure a corrective action plan, if applicable, is implemented and sustained.

End of report.